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AU PAIR & ELDERLY CARE GIVING

A Comprehensive Course

2023-2024



MODULE FOUR: PALLIATIVE CARE.

After completing this module, you will be able to:

- Understand palliative care.
- Take care of terminally ill patients.
- Provide safe and caring environment.
- Respect attentively for the patient and family.
- Provide a comfortable and pain-free environment.



WHAT IS PALLIATIVE CARE?

Palliative care (WHO, 2002a) is an approach that improves the quality of life of patients and their families facing the problems associated with life-threatening illness, through the prevention and relief of suffering by means of early identification and impeccable assessment, and treatment of pain and other problems – physical, psychosocial and spiritual.

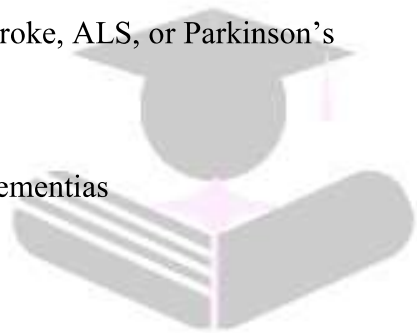
Palliative care is both a philosophy of care and an organized, highly structured system for delivering care. The goal of palliative care is to prevent and relieve suffering and to support the best possible quality of life for patients and their families, regardless of the stage of the disease or the need for other therapies. Palliative care expands traditional disease-model medical treatments to include the goals of enhancing quality of life for patients and family members, helping with decision making, and providing opportunities for personal growth. Palliative care can be rendered along with life-prolonging treatment or as the main focus of care.

When patients choose to begin palliative care, they receive a formal assessment of their health early in the process. Symptoms most commonly addressed include:

- Pain or discomfort
- Shortness of breath
- Fatigue
- Anxiety
- Depression
- Lack of appetite
- Nausea
- Constipation
- Adjusting to and living with the diagnosis of a serious health condition
- Sleep problems.

The most common health conditions addressed in palliative care include:

- Cancer
- Congestive heart failure (CHF)
- Kidney failure
- Liver failure
- Chronic obstructive pulmonary disease (COPD) or other lung diseases
- HIV/AIDS
- Spinal cord injuries
- Brain diseases such as stroke, ALS, or Parkinson's
- Multiple sclerosis (MS)
- Alzheimer's and other dementias



Goals of Palliative Care:

- Provide relief from pain and other distressing symptoms.
- Affirm life and regards dying as a normal process.
- Intend neither to hasten nor postpone death. Integrate the psychological and spiritual aspects of patient care.
- Offer a support system to help patients live as actively as possible until death.
- Offer a support system to help the family cope during the patient's illness and in their own bereavement.
- Use a team approach to address the needs of patients and their families, including bereavement counseling, if indicated.
- Enhance quality of life, which may also positively influence the course of illness.

WHO PROVIDES PALLIATIVE CARE?

Palliative care is typically carried out by a team of professionals who can address multiple concerns. It may be offered by hospitals, home care agencies, cancer treatment centers, and long-term care facilities. Palliative care can be provided by a team of healthcare professionals, which may include:

1. **Palliative care physicians:** Physicians who specialize in palliative care and have expertise in managing symptoms, addressing psychological and spiritual needs, and coordinating care with other healthcare providers.
2. **Nurses:** Nurses who specialize in palliative care work closely with the patient, family, and healthcare team to provide comprehensive care, manage symptoms, and address emotional and spiritual needs.
3. **Caregivers:** They attend to the patient's day to day needs including; hygiene, toileting, bathing, taking medication, physical needs, emotional support and many more.
4. **Social workers:** Social workers can provide emotional and practical support to patients and their families, helping them navigate the healthcare system, connect with resources, and address financial concerns.
5. **Chaplains:** Chaplains are trained in providing spiritual support to patients and families, and can help address issues related to meaning and purpose.
6. **Pharmacists:** Pharmacists can play an important role in palliative care by managing medications, providing education about potential side effects, and monitoring drug interactions.
7. **Other healthcare providers:** Palliative care may also involve other healthcare providers, such as physical therapists, occupational therapists, and speech therapists, who can help patients maintain their physical function and quality of life.

It's important to note that palliative care can be provided in a variety of settings, including *hospitals, long-term care facilities, hospice, and in the patient's home*. The specific healthcare

professionals involved in providing palliative care may vary depending on the setting and the needs of the patient.

PRINCIPLES OF PALLIATIVE CARE

Palliative care aims to relieve **physical pain** and **psychological, social, and spiritual suffering** while supporting the patient's treatment goals and respecting the patient's racial, ethnic, religious, and cultural values. Like all good patient care, palliative care is based on the *fundamental ethical principles of autonomy, beneficence, no maleficence, justice, and duty*.

Although palliative care includes hospice care and care near the time of death, it also embraces the management of pain and suffering in medical and surgical conditions throughout life. If palliation is taken to apply solely to care near the time of death, or "comfort measures only," it fails to include the life-affirming quality of active, symptomatic efforts to relieve the pain and suffering of individuals with chronic illness and injury. In this respect, palliative care is required in the management of a broad range of surgical patients and is not restricted to those at the end of life.

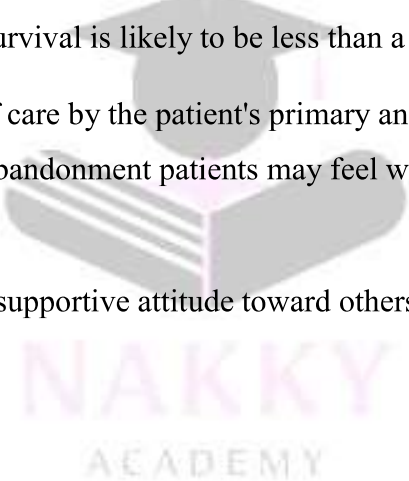
The four cardinal principles are;

- Non-maleficence (Do no harm)
- Beneficence (Do good)
- Patient autonomy (patient's right to be informed and involved in decision making)
- Justice (balancing needs of individuals with that of society)

The four cardinal principles need to be applied against a background of;

1. Respect the dignity and autonomy of patients, patients' surrogates, and caregivers.
2. Honor the right of the competent patient or surrogate to choose among treatments, including those that may or may not prolong life.
3. Communicate effectively and empathically with patients, their families, and caregivers.

4. Identify the primary goals of care from the patient's perspective, and address how the surgeon's care can achieve the patient's objectives.
5. Strive to alleviate pain and other burdensome physical and nonphysical symptoms.
6. Recognize, assess, discuss, and offer access to services for psychological, social, and spiritual issues.
7. Provide access to therapeutic support, encompassing the spectrum from life-prolonging treatments through hospice care, when they can realistically be expected to improve the quality of life as perceived by the patient.
8. Recognize the physician's responsibility to discourage treatments that are unlikely to achieve the patient's goals, and encourage patients and families to consider hospice care when the prognosis for survival is likely to be less than a half-year.
9. Arrange for continuity of care by the patient's primary and/or specialist physician, alleviating the sense of abandonment patients may feel when "curative" therapies are no longer useful.
10. Maintain a collegial and supportive attitude toward others entrusted with care of the patient.



DOMAINS OF PALLIATIVE CARE

